

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY IN TEXAS

A Texas-licensed Home and Community Support Services Agency (HCSSA) in the Personal Assistance Services category — providing non-medical personal care, homemaker, and companion services so seniors and people with disabilities can live safely at home.

A step-by-step startup and operations guide

Licensing (HHSC) · Attendants & training · Funding & billing · Operations

Regulatory basis: Texas Health and Safety Code Chapter 142 and 26 Texas Administrative Code, Chapter 558 (Home and Community Support Services Agencies), Personal Assistance Services category; licensed by the Texas Health and Human Services Commission (HHSC). Funding: private pay; Texas Medicaid personal attendant services (Primary Home Care, Community Attendant Services, Family Care, Community First Choice, and STAR+PLUS); Veterans' benefits; and long-term-care insurance.

How to Use This Guide

This guide walks you through opening and operating a non-medical home care agency in Texas — a Home and Community Support Services Agency (HCSSA) licensed in the Personal Assistance Services (PAS) category. A PAS agency provides personal care, homemaker, and companion services (help with bathing, dressing, meals, housekeeping, errands, transportation, medication reminders, and companionship) so clients can stay safely in their own homes. It is the accessible, high-demand side of home care: no Certificate of Need, no clinical staff required, and a fast-growing market as the Texas senior population expands.

Important — non-medical means non-medical

A PAS agency is a **non-medical** business. Your attendants assist with daily living, homemaking, companionship, and self-administered medication — and health-related tasks only when a nurse has delegated them (or determined they don't require delegation) under the Board of Nursing rules. They do not provide skilled nursing, therapy, or medication administration. Skilled care requires a different HCSSA category (Licensed Home Health) and, to bill Medicare, Medicare certification. This guide is an educational resource, not legal, tax, or billing advice — verify every step and figure with HHSC and the relevant payer before you act.

The big picture, in one paragraph

Here is the whole business in a nutshell. You license your agency with HHSC as an HCSSA in the Personal Assistance Services category. You hire attendants who are background-checked (through DPS and FBI fingerprinting plus the Employee Misconduct Registry and Nurse Aide Registry) and trained to competency. You find clients through hospitals, senior communities, case managers, managed-care plans, and families. And you get paid — mostly private pay to start, plus Texas Medicaid personal attendant programs, Veterans' benefits, and long-term-care insurance as you grow. License it, staff it, fill it, bill it.

The path at a glance — step by step

1. Form your business (LLC or corporation) and get an EIN.
2. Set up a physical place of business with secure record storage (not a virtual address).
3. Designate a qualified administrator and an alternate administrator, and complete the required administrator training.
4. Build your Policy & Procedure Manual, attendant handbook and training plan, and forms — you need these for the application.
5. Put insurance in place; if you'll bill Medicaid, plan for the surety bond and provider enrollment.
6. Complete DPS/FBI, Employee Misconduct Registry, and Nurse Aide Registry checks on owners, the administrator, and attendants.
7. Apply for your HCSSA (PAS) license through the TULIP portal and pay the fee.
8. Pass the pre-survey / initial survey and receive your license.
9. Recruit, screen, train, and supervise your attendants.
10. Enroll with Texas Medicaid (TMHP/PEMS) and contract with STAR+PLUS plans as you add Medicaid clients — and start referral relationships early.

Part 1 — What This Business Is

A Texas Personal Assistance Services agency provides non-medical home care — assistance with activities of daily living (bathing, dressing, grooming, toileting, transferring, mobility, and eating) and instrumental, homemaker, and companion services (meal preparation, light housekeeping, laundry, shopping, errands, transportation, medication reminders, supervision, and companionship) — delivered by attendants in clients' homes. It does not provide skilled nursing or therapy.

License: Home and Community Support Services Agency (HCSSA), Personal Assistance Services category, issued by HHSC under Texas Health and Safety Code Chapter 142 and 26 TAC Chapter 558.

With that license you can employ attendants and provide non-medical services for pay from your licensed place of business — the license is issued per place of business, so each location from which you direct services needs its own. What you can't do is provide or advertise skilled nursing, therapy, or medication administration; the moment a client needs those, you coordinate with a licensed home health agency, a physician, or home health rather than stretching your scope. Staying clearly on the non-medical side of that line is both a legal requirement and a simpler, cleaner business to run.

Why this is a strong business to start

Non-medical home care is the accessible door into a huge and fast-growing market. Texas has one of the fastest-growing senior populations in the nation — the state's elder population is projected to reach roughly 5.9 million, about one in five Texans, by 2030, growing faster than the national rate. Unlike skilled home health, a PAS agency needs no Certificate of Need and no clinical staff, and Texas's open-market approach and favorable operating economics make it one of the more accessible states in which to launch. What you are building is a trusted, reliable service: safe, screened, trained attendants who show up and help people remain in their own homes. That reliability, and your reputation, is what you are really selling.

The demographics are on your side. Older adults are the fastest-growing age group in Texas, most people strongly prefer to age at home rather than in a facility, and the supply of quality attendants has not kept up with demand — which is why hospitals, families, case managers, and managed-care plans are actively looking for reliable agencies. The flip side is that it is a real business with real obligations — licensing, labor law, documentation, supervision, and payroll — so it rewards operators who build good systems and take compliance seriously, which is exactly what this package is designed to help you do.

You can start focused (a particular metro, a particular niche such as dementia support or post-hospital care) and grow. Many successful agencies begin largely private-pay and add Texas Medicaid attendant clients — through Primary Home Care, Community Attendant Services, and STAR+PLUS — as they build the systems to serve them.

There is also room to grow into adjacent lines once you are established. Many agencies begin with non-medical PAS and later add the Medicaid attendant programs, then eventually pursue the Licensed Home Health category or Medicare certification to offer skilled services — a much larger clinical undertaking, but one your non-medical agency's reputation and referral relationships can feed. Starting non-medical is the sensible on-ramp: it is achievable, it generates cash flow and referral relationships quickly, and it teaches you the operational discipline every later expansion depends on. Build this well first, and the bigger opportunities become far easier to reach.

Who your clients are

It helps to picture who you serve. Some are seniors who want to stay in their own homes and need help with bathing, meals, and companionship a few hours a day. Some are recovering from a hospital stay or surgery and need short-term support so they don't end up readmitted. Some have dementia and need supervision and a calm, familiar routine. Some are younger adults with disabilities directing their own care. Each comes with a different funding path (Part 6) and a different level of need, but all need the same core thing: a dependable, screened, trained attendant who treats them with respect. Deciding early which of these you serve best — and building your roster and marketing around them — is how you stand out.

Getting a realistic picture of the market

Go in with clear eyes. Non-medical home care has a low clinical barrier but a real operational one, and in a growing metro you will have competitors — national franchises, regional agencies, and independents. What wins is not being first but being reliable: attendants who show up on time, communicate, and are matched well to the client, backed by an owner who answers the phone and solves problems. Because the work is local and relationship-driven, a focused new agency can absolutely take share from a large, impersonal one. Pick a lane — a specific part of the metro, a niche such as dementia support, post-hospital recovery, or veterans — and build your roster,

training, and marketing around it, rather than trying to be everything to everyone on day one. Depth in one niche produces referrals; being generic produces silence.

Think, too, about your payer mix over time. Private pay funds the business immediately and carries the lightest administrative load, so most agencies start there and use it to build systems and reputation. Texas Medicaid attendant programs — Primary Home Care, Community Attendant Services, and STAR+PLUS — open a large, steady population of clients, but they require enrollment, contracting, EVV, and disciplined documentation, so agencies typically add them once the basics are running. Veterans' programs and long-term-care insurance round out the mix. A deliberate sequence — private pay first, Medicaid as you mature — keeps cash flowing while you build the muscle to handle payer requirements.

Part 2 — Who Regulates You

- **HHSC Long-Term Care Regulatory** — your licensing regulator; it approves your HCSSA (PAS) license, sets the Chapter 558 standards, and surveys you. Licensing and certification: 512-438-2630.
- **Texas Board of Nursing** — its rules on RN delegation to unlicensed personnel (22 TAC Chapter 225) govern which health-related tasks your attendants may perform.
- **Texas Medicaid & Healthcare Partnership (TMHP)** — the gateway for Medicaid provider enrollment (PEMS) and fee-for-service claims; STAR+PLUS is billed through the member's managed-care organization.
- **DFPS / Adult Protective Services** — where abuse, neglect, or exploitation of adults in the community is reported (Texas Abuse Hotline 1-800-252-5400); you also self-report allegations to HHSC.
- **EVV (HHAeXchange)** — the state Electronic Visit Verification system you must use for Medicaid personal-care visits.

One distinction worth internalizing early: licensing and funding are separate tracks on separate timelines. You can hold a valid HCSSA license and not yet be enrolled with Texas Medicaid, and you can be deep in a payer conversation while your license is still pending. Sequence them deliberately — you generally need the license first, then you pursue the HHSC contract, TMHP/PEMS enrollment, and STAR+PLUS managed-care contracts — and start those conversations early, because they move slower than new owners expect.

The delegation rules deserve special attention because they define the edge of your scope. Under the Texas Board of Nursing rules (22 TAC Chapter 225), certain health-related tasks an attendant might do — for example, assistance that touches on nursing — must be delegated by a registered nurse or determined by an RN not to require delegation. Your job is to build a clear written policy so that everyone knows which tasks require RN involvement, and to make sure an RN is actually in the loop where the rules require it. This is not bureaucratic box-checking: delegation exists to protect the client and the attendant by ensuring a nurse has judged a specific task appropriate for a specific client and attendant, with the right teaching and oversight. Get this framework right and you can serve clients with meaningful needs while staying squarely within the non-medical license.

Part 3 — Business, Office, and Insurance Setup

- **Legal entity + EIN:** form an LLC or corporation with the Texas Secretary of State, get your federal EIN, and register with the Texas Comptroller.
- **Physical place of business:** a real office with secure storage for confidential client and personnel records — not a virtual address; HHSC may inspect it.
- **Liability insurance:** general and professional liability appropriate to home care, provided with your application and kept current.
- **Medicaid surety bond:** if you enroll as a Texas Medicaid provider, you must maintain the required surety bond in PEMS.
- **Workers' compensation:** Texas does not require it — but if you don't subscribe, you must follow the non-subscriber notice and reporting rules (see the callout).

What each of these really means

The entity is your legal shield: form an LLC or corporation so the business holds the license and the liability, and never commingle business and personal money. The office is a real requirement, not a formality — HHSC expects a secure place for confidential records and may inspect it. Insurance is not one policy: you need general and professional liability, the Medicaid surety bond if you bill Medicaid, and — if you drive clients — commercial auto. Work with a broker who knows home care, because standard small-business policies often miss the risks you actually carry: attendants working alone in clients' homes, transportation of clients, and allegations of abuse or theft.

Keep your compliance calendar in view from the start. Beyond the HCSSA license and its three-year renewal, you will track the franchise-tax and any sales-tax filings with the Texas Comptroller, your insurance and bond renewals, background-check and registry re-checks, administrator continuing education, EVV training, and — if you enroll in Medicaid — provider revalidation deadlines. None of these is hard individually, but missing one can interrupt your ability to operate or bill, so assign responsibility for each, put the dates on a calendar, and review them monthly. A simple tracking habit prevents the avoidable lapses that quietly damage new agencies.

Workers' comp is optional in Texas — but read this

Texas is unusual: it **does not require most employers to carry workers' compensation**. If you choose not to subscribe, you must notify employees and the Texas Department of Insurance, Division of Workers' Compensation, and file the required reports — and you lose certain legal protections if an attendant is injured. Because attendants face real field risks (lifting injuries, falls, travel), weigh coverage carefully rather than defaulting to 'no.' Confirm current requirements before you decide.

A few Texas-specific setup notes save headaches later. Register your entity with the Secretary of State, get your EIN, and set up with the Texas Comptroller — Texas has no state income tax, but you will deal with the franchise tax and, depending on your model, sales-tax registration. Choose your office with the referral relationship in mind: a professional address signals credibility to discharge planners and case managers, keeps your home address private, and gives you the secure, inspectable record storage HHSC expects. Whatever you choose, lock down record security early — it is something a surveyor looks at, and it protects your clients' confidential information.

On insurance, use a broker who actually understands home care and tell them plainly what your attendants will do — work alone in clients' homes, assist with transfers and personal care, and sometimes transport clients. Describing your agency as an ordinary office business leaves you underinsured exactly where the risk lives. You will generally want general and professional liability, commercial auto if attendants drive clients, the Medicaid surety bond once you enroll, and a clear, written decision on workers' compensation given Texas's non-subscriber rules. Get these quotes early; a certificate of insurance is part of what you submit, and waiting until the end can stall your application.

Part 4 — Get Licensed: The HCSSA (PAS) Process

1. Form your entity and get your EIN; set up your physical office with secure record storage.
2. Designate your administrator and alternate administrator — each meeting the § 558.244 qualifications — and complete the administrator training required by § 558.259.
3. Prepare your Policy & Procedure Manual, attendant handbook and training/competency plan, and forms — your policies are part of the application.
4. Complete DPS and FBI fingerprint checks plus Employee Misconduct Registry and Nurse Aide Registry checks on owners, the administrator, and staff.
5. Put liability insurance in place (and plan the surety bond if you'll bill Medicaid).
6. Submit your HCSSA (PAS) application through the TULIP portal and pay the fee.
7. Complete the pre-survey / initial survey and correct any deficiencies.
8. Receive your license and begin operating within the PAS scope.

The numbers to budget

Confirm the current HCSSA license fee in HHSC's published fee schedule (Provider Letter 2021-03) through the TULIP portal — the license term is **three years**, and routine change reports carry a \$30 fee (a \$100 late fee applies if not reported timely). If you'll bill Medicaid, the Texas Medicaid **provider enrollment application fee is \$750 for 2026**. Processing typically runs **60–90 days** (allow up to 120). Texas requires no Certificate of Need.

Where your manual, handbook, and forms fit. Texas requires you to submit a comprehensive policy and procedure manual as part of licensure. Your Policy & Procedure Manual, your attendant handbook and training plan, and your Forms Packet are built to Chapter 558 — have them ready before you apply, because a generic template downloaded from the internet is a common reason applications stall.

What review and survey look at — and where agencies lose time

The things that most often delay a first license are avoidable: an incomplete TULIP application, missing background clearances or registry checks, insurance or bond not yet in place, an administrator who hasn't completed required training, a training/competency plan that doesn't match the tasks attendants will perform, and policies that are generic rather than built to Texas's rules. Several pieces depend on outside parties (fingerprint results, insurance certificates) and can't be rushed at the end, so start early. If HHSC issues a deficiency, respond fully and promptly — partial responses and long gaps are what stretch a 60-day approval into a 120-day one.

The administrator is the center of gravity for your license. This is the person HHSC holds accountable for the agency's daily operation and Chapter 558 compliance — for making sure every attendant is screened and competent before serving a client, that service plans and supervision happen, that incidents and abuse are reported, and that the quality program runs. Choose your administrator and alternate carefully, make sure both meet the § 558.244 qualifications and complete the § 558.259 training, and treat the role as real rather than a name on a form. When your administrator is strong, surveys go smoothly and problems get caught early; when the role is neglected, small compliance gaps compound.

Set your timeline expectations honestly. Between assembling your policy manual and forms, completing background and registry checks, putting insurance in place, and passing the survey, a first HCSSA license commonly takes a couple of months — and longer if pieces arrive late or a deficiency notice bounces back and forth. Build a checklist, start the outside-dependent items (fingerprints, insurance, bond) first, and keep your application complete and consistent. The agencies that open fastest are simply the ones that submit a clean, complete packet and respond to HHSC promptly and fully.

Part 5 — Your Attendants

Your attendants are the business. In Texas they are unlicensed personnel providing personal assistance services, and Chapter 558 is specific about them:

- **Eligibility:** at least 18 — or under 18 if a high school graduate or enrolled in a vocational education program and competency is demonstrated (§ 558.404(c)).
- **Screening:** DPS and FBI fingerprint checks plus Employee Misconduct Registry and Nurse Aide Registry checks before any contact with a client.
- **Training:** competency-based — Texas sets no single statutory hour count for PAS attendants; you define and validate competency for the tasks assigned.
- **Supervision:** by a supervisor who is a licensed nurse or has two years of college (§ 558.404(g)), with client input on frequency.
- **Health-related tasks:** performed only under RN delegation or an RN determination that delegation isn't required (22 TAC Chapter 225).

Screening deserves real rigor because it is your primary client safeguard. Before anyone has contact with a client, complete the DPS and FBI fingerprint-based criminal history check and verify the person is not listed on the Employee Misconduct Registry or barred by the Nurse Aide Registry. A disqualifying finding means the person

cannot work in a client-contact role — there is no discretion to ‘make an exception.’ Re-check the registries as required, act immediately on any later finding, and keep the documentation, because a surveyor will look for proof that every attendant was cleared before their first visit. Attendants work alone with people who may be frail, cognitively impaired, or unable to report mistreatment, so this is exactly where cutting corners does the most harm.

Training that prepares attendants, not just checks a box

Because Texas doesn’t hand you a fixed curriculum, the quality of your training is up to you — and it is a real differentiator. The tasks that actually keep clients safe reward genuine training: safe transfers and fall prevention, recognizing and reporting a change in condition, dementia care, professional boundaries and abuse recognition, infection prevention, and your documentation and EVV systems. Validate competency (not just attendance), re-train after any concern, and keep the records — your training plan is part of your license application and your competency records are something a surveyor checks. Good training is also a retention tool: attendants who feel competent and supported stay.

Supervision and delegation

Supervision is how you maintain quality at a distance, since attendants work unsupervised in the home. Use a qualified supervisor (a nurse or someone with two years of college, per § 558.404(g)), get client input on how often supervision happens, and use supervisory visits and documentation review to confirm care matches the service plan and to catch problems early. Where an attendant performs any health-related task, an RN must delegate it or determine it doesn’t require delegation under 22 TAC Chapter 225 — this protects the client and keeps the attendant within scope. Attendants assist with, but never administer, medication.

It helps to understand how clients qualify for the Medicaid programs, because that shapes who your case-manager partners send you. Primary Home Care and Community Attendant Services serve Medicaid-eligible people with functional limitations in activities of daily living who need help to remain safely at home; eligibility and the amount of authorized service are determined by the state or the managed-care organization, not by your agency. STAR+PLUS members have their services authorized in an individual service plan built by the MCO’s service coordinator, so your job is to deliver exactly what the ISP authorizes and to communicate changes back to the coordinator. You don’t determine eligibility — but knowing how it works lets you speak your referral partners’ language and set client expectations accurately.

Recruiting, wages, and keeping good attendants

Your hardest ongoing job is not getting clients — it is recruiting and keeping good attendants, because the caregiver shortage is real and turnover is the industry’s biggest cost. Texas attendant wages are modest (Medicaid attendant rates as of late 2025 support an average wage around \$13/hour, with private pay often supporting more), so you compete on more than pay: respect, reliable scheduling, good matching, real training, quick support when something goes wrong, and treating attendants as professionals. An agency that attendants want to work for has better coverage, fewer missed visits, happier clients, and more referrals — the whole business runs on it. Build your screening, onboarding, and supervision so a good attendant’s first weeks are organized and supported, not chaotic.

Coverage discipline matters as much as recruiting. A missed or late visit is not just a service failure — for a frail client it can be a safety event, and for a Medicaid visit it shows up in your EVV data and your quality metrics. Build a scheduling system with backup coverage, clear call-out procedures, and a supervisor who can step in, and track missed and late visits from day one so you can fix the causes. Reliability is the single thing clients, families, and referral sources judge you on.

Part 6 — How You Get Paid

Most agencies start largely private pay — clients or families paying hourly — because it requires no enrollment and funds the business while you build. As you grow, Texas Medicaid personal attendant programs, Veterans’ benefits, and long-term-care insurance expand who you can serve.

Funding sources

Source	What it is	How to access
Private pay	Clients or families pay hourly; no enrollment required.	Set clear rates in your service agreement; bill directly.
Primary Home Care (PHC) / Community Attendant Services (CAS)	Texas Medicaid non-technical attendant services for people with functional limitations (26 TAC Ch. 277).	Hold an HCSSA (PAS) license + an HHSC contract; enroll in TMHP/PEMS.
Family Care	Title XX attendant services for eligible clients.	HHSC contract; delivered under Chapter 277.
Community First Choice (CFC)	Medicaid state-plan attendant services and habilitation.	Delivered through STAR+PLUS; contract with the member's MCO.
STAR+PLUS	Mandatory managed long-term services and supports; the MCO authorizes services in the member's ISP.	Contract with the STAR+PLUS MCOs in your service area; bill the MCO.
Veterans' benefits	VA programs that pay for in-home personal care for eligible veterans.	Coordinate with the VA and the veteran's care team.
Long-term-care insurance	Private policies that reimburse home care.	Verify the client's policy terms and bill per its rules.

Billing, enrollment & EVV

To bill Texas Medicaid you generally need three things in sequence: your HCSSA (PAS) license, an HHSC contract meeting the contracting requirements of 26 TAC § 277.11, and enrollment in the Texas Medicaid & Healthcare Partnership Provider Enrollment and Management System (PEMS) — where you also maintain the required surety bond. STAR+PLUS services are authorized through the member's individual service plan and billed through the member's managed-care organization; fee-for-service Primary Home Care and Community Attendant Services are billed through TMHP. All Medicaid personal-care visits must be captured in Electronic Visit Verification through HHAExchange.

Program	Billing path	Codes & notes
STAR+PLUS PAS	Through the member's MCO, under the authorized ISP.	Use current HCPCS codes/modifiers from the LTSS Codes & Modifiers matrix (STAR+PLUS Handbook, Appendix XVI).
Respite (STAR+PLUS)	Through the member's MCO.	S5151 (respite) per the current matrix; modifiers per HHSC.
PHC / CAS (fee-for-service)	Through TMHP.	Per the Texas Medicaid fee schedule and Chapter 277.

A note on rates and codes

Texas is mid-transition: on Sept. 1, 2025 HHSC moved from the RUG-III model to the Patient-Driven Payment Model (PDPM), and codes and modifiers change as each member's ISP renews — claims filed with old codes are denied. Always use the **current LTSS Codes & Modifiers matrix (Appendix XVI)** as your authoritative source. As a planning anchor, attendant-service rates effective 9/1/2025 support an average attendant wage of about \$13.00/hour. Verify current codes and rates with HHSC and your MCOs before you bill.

Two practical cautions on the money side. First, private-pay pricing: set a rate that covers your fully-loaded costs — attendant wage, payroll taxes and benefits, workers' comp if you carry it, supervision, insurance, office, and a margin — not just a number that sounds competitive. Underpricing is one of the most common ways new agencies

fail, because the thin margin can't absorb turnover, training, or a slow month. Second, sequence Medicaid deliberately: the HHSC contract, TMHP/PEMS enrollment, surety bond, and MCO contracts each take time, so start them well before you need the revenue, and do not deliver 'billable' Medicaid services until enrollment is actually complete — services delivered before your effective date generally can't be billed.

Make EVV a strength, not an afterthought. From your very first Medicaid visit, capture visits correctly through HHAeXchange, train attendants to clock in and out properly, and monitor exceptions so you can fix them through visit maintenance before they become denied claims. Clean EVV and documentation are what let you pass a payer audit and get paid on time — and building the habit early is far easier than retrofitting it after problems pile up.

Alongside referral relationships, build a basic but credible presence so that families searching for care can find and trust you. A clean website that explains your services and service area, a complete Google Business Profile, and a handful of genuine reviews do a lot of quiet work — discharge planners and families check you out online before they call. Keep your messaging honest and non-medical (personal care, companionship, homemaking, help staying safe at home), avoid any claim that sounds like skilled care, and make it effortless to reach you. You are selling trust and reliability; every touchpoint, from your voicemail greeting to your intake call, should reinforce that you are organized, responsive, and professional. In a referral-driven business, being easy to find and easy to trust turns interest into clients.

Veterans and long-term-care insurance

Two funding sources are worth cultivating deliberately as you grow. Veterans' programs can pay for in-home personal care for eligible veterans and surviving spouses, and veterans are an underserved, loyal client population; building a relationship with your regional VA and veteran service organizations opens a steady referral channel. Long-term-care insurance reimburses home care for clients who hold policies — verify each policy's terms (elimination period, daily benefit, covered services, documentation requirements) before you commit, and bill exactly as the policy requires, because these payers deny sloppy claims. Neither source replaces private pay or Medicaid, but together they widen the range of clients you can serve and smooth out your revenue.

Part 7 — Find Your Clients

Referrals are how home care agencies actually grow. Your best sources are the people who already have your future clients in front of them: hospital discharge planners and case managers, skilled-nursing facilities, physicians and clinics, your regional Area Agency on Aging and Aging & Disability Resource Center, STAR+PLUS managed-care service coordinators, senior living communities, and families. Get on hospital discharge lists, become a contracted provider with the STAR+PLUS plans in your area, and build genuine relationships with the case managers who place clients.

Build referrals — legally

You can earn referrals, but you cannot **pay** for them when clients are funded by Medicaid or Medicare — paying for such referrals violates the federal Anti-Kickback Statute. Build relationships and get contracted; don't pay per client. (Your Referral System resource covers the official pathways and the legal line in detail.)

Not all referral sources are equal, so focus your energy. Hospital discharge planners and skilled-nursing case managers send clients who need care now; Area Agencies on Aging and Aging & Disability Resource Centers connect you to the aging network and to families searching for help; STAR+PLUS service coordinators route Medicaid members to contracted agencies; and physicians, senior communities, and elder-law attorneys refer steadily once they trust you. Pick a focused 'target list' of the highest-value sources near you, show up consistently, make it easy to refer to you, and — above all — deliver so well that they refer again. One reliable referral relationship, nurtured over time, is worth more than a hundred cold introductions.

Part 8 — Operate Well: The First 90 Days

- **Documentation and EVV from day one:** set up clean visit documentation and HHAEExchange EVV before your first Medicaid visit — retrofitting later is painful.
- **Supervision cadence:** start supervisory visits and documentation review immediately; don't wait for a problem.
- **Stand up QAPI:** begin tracking complaints, incidents, missed/late visits, and EVV exceptions from the first week.
- **Survey readiness:** keep records organized and practice matched to policy — the initial survey confirms rather than surprises when you do.
- **Referral rhythm:** make outreach to discharge planners and case managers a weekly habit, not a launch event.

Picture the first ninety days concretely. You license the agency, then hire your first two or three attendants — screening them fully, training them to competency, and pairing them with your earliest clients (often private pay). From the first visit you run clean documentation and, for any Medicaid client, EVV through HHAEExchange. Your administrator conducts early supervisory visits, not because anything is wrong but because that is how you catch small issues before they grow. You start your quality log immediately — tracking complaints, incidents, and missed or late visits — and you review it weekly. And you spend part of every week on referral relationships: a visit to a discharge planner, a call to a case manager, a coffee with a senior-community director. Ninety days in, the goal is not size but a working system: attendants who are supported, clients who are safe and satisfied, documentation that would pass a survey, and a referral rhythm that keeps new clients coming. Build the system first, and growth follows.

A last piece of framing: treat compliance and quality as the same thing, not competing priorities. Nearly every Chapter 558 requirement — screening, training, supervision, service planning, documentation, incident reporting — exists because it protects a client, and doing it well is also what makes clients and referral sources trust you. Agencies that see compliance as paperwork to minimize tend to struggle on both fronts; agencies that build genuine systems tend to pass surveys easily and grow steadily. This package — the policy manual, the attendant handbook, the forms, and this guide — is designed to give you those systems from day one.

Common Mistakes to Avoid

- **Crossing the non-medical line** — having attendants perform nursing tasks or administer medication outside RN delegation.
- **Applying with generic policies** — a P&P manual not built to Chapter 558 is a common reason applications stall.
- **Skipping a registry check** — missing the Employee Misconduct Registry or Nurse Aide Registry check before client contact.
- **Treating training as attendance** — not validating competency for the tasks attendants actually perform.
- **Billing before enrollment** — delivering 'billable' Medicaid services before your HHSC contract and PEMS enrollment are complete.
- **Using old billing codes** — filing with pre-PDPM codes after an ISP renews, which gets claims denied.
- **Defaulting to no workers' comp** — without understanding the non-subscriber obligations and risks.

Two Worked Examples

Example 1 — Houston (Harris County)

You form your LLC, set up a small office in Houston with secure record storage, and designate yourself as administrator (completing the required administrator training) with a qualified alternate. You build your Chapter 558 policy manual, training plan, and forms, complete DPS/FBI, Employee Misconduct Registry, and Nurse Aide Registry checks, put liability insurance in place, and apply through TULIP. While your license is pending, you

connect with the aging network: the Houston-Galveston Area Agency on Aging at (713) 627-3200 and the Harris County Aging & Disability Resource Center at (832) 393-5564 for the surrounding metro, and 2-1-1 Texas for local resources. You introduce yourself to discharge planners at nearby hospitals and skilled-nursing facilities. Once licensed, you start with private-pay clients, then enroll in TMHP/PEMS and contract with the STAR+PLUS plans serving the Houston service area to add Medicaid attendant clients. For licensing questions you call HHSC at 512-438-2630.

Example 2 — San Antonio (Bexar County)

You follow the same path in San Antonio. You lean on the Alamo Area Agency on Aging at (210) 362-5200 and the Bexar Area Agency on Aging / ADRC at (210) 477-3275 to plug into the local aging network and referral sources, and 2-1-1 Texas for services. You build relationships with case managers at the major hospital systems and with the STAR+PLUS service coordinators in the Bexar service area. You start private-pay, keep your EVV and documentation clean from the first visit through HHAExchange, and add Medicaid clients once your HHSC contract and PEMS enrollment are complete. When you're ready to grow, you repeat the pattern in Austin — Area Agency on Aging of the Capital Area, (512) 916-6062 — or Dallas.

Find Your Own Local Numbers

Your strongest relationships are local, so build your own contact list for your service area:

1. Call 2-1-1 Texas (dial 2-1-1) or the statewide HHS information line at 855-937-2372 to find your county's Area Agency on Aging and Aging & Disability Resource Center.
2. Identify the hospitals and skilled-nursing facilities in your service area and ask each for the case-management / discharge-planning department.
3. Find the STAR+PLUS managed-care organizations that serve your Medicaid service delivery area — these are your payer and referral partners.
4. Locate your HHSC Long-Term Care Regulatory regional office for survey and policy questions.
5. Verify every number and program before you rely on it — contacts and rules change.

Who to Call — Texas Contacts

For...	Who to contact
HHSC Licensing & Certification (HCSSA)	512-438-2630
HHSC Policy & Rule Interpretations	512-438-3161
Texas Medicaid & Healthcare Partnership (TMHP) — provider enrollment	tmhp.com
Statewide HHS / ADRC information line	855-937-2372
2-1-1 Texas (local health & human services)	Dial 2-1-1
Texas Abuse Hotline (DFPS / Adult Protective Services)	1-800-252-5400
Houston-Galveston Area Agency on Aging	713-627-3200
Harris County Aging & Disability Resource Center	832-393-5564
Dallas Area Agency on Aging	214-871-5065
North Central Texas Area Agency on Aging (Fort Worth region)	800-272-3921
Alamo Area Agency on Aging (San Antonio)	210-362-5200

For...	Who to contact
Bexar Area Agency on Aging / ADRC (San Antonio)	210-477-3275
Area Agency on Aging of the Capital Area (Austin)	512-916-6062

Glossary

- **HCSSA** — Home and Community Support Services Agency; the Texas license under which home care is provided.
- **PAS** — Personal Assistance Services; the non-medical HCSSA license category.
- **TULIP** — Texas Unified Licensure Information Portal, where you apply and report.
- **EMR / NAR** — Employee Misconduct Registry and Nurse Aide Registry, checked before client contact.
- **ISP** — Individualized Service Plan; the signed plan of services for each client.
- **STAR+PLUS** — Texas’s mandatory managed long-term services and supports program.
- **PHC / CAS** — Primary Home Care and Community Attendant Services; Medicaid attendant programs.
- **EVV** — Electronic Visit Verification; Texas uses HHAExchange.
- **PEMS** — TMHP’s Provider Enrollment and Management System.
- **PDPM** — Patient-Driven Payment Model; Texas’s current LTSS payment methodology (since 9/1/2025).

Core Rules & References

- **Texas Health and Safety Code, Chapter 142** — the HCSSA statute.
- **26 TAC Chapter 558** — Licensing Standards for HCSSAs, including PAS standards (§ 558.404).
- **26 TAC Chapter 277** — Primary Home Care, Community Attendant Services, and Family Care programs.
- **22 TAC Chapter 225** — Board of Nursing rules on RN delegation to unlicensed personnel.
- **STAR+PLUS Handbook, Appendix XVI** — the LTSS Codes & Modifiers billing matrix.
- **Texas EVV Policy Handbook** — Electronic Visit Verification requirements (HHAExchange).

Your Startup Checklist

Use this as a running checklist as you open. Verify current details with HHSC and the relevant payer as you go.

1. Form your LLC/corporation, get your EIN, and register with the Texas Comptroller.
2. Secure a physical office with locked, confidential record storage.
3. Designate your administrator and alternate; complete required administrator training.
4. Build your Chapter 558 Policy & Procedure Manual, attendant handbook and training/competency plan, and forms.
5. Put general and professional liability insurance in place; decide on workers’ compensation and handle non-subscriber steps if you decline it.
6. Complete DPS/FBI, Employee Misconduct Registry, and Nurse Aide Registry checks on owners, administrator, and staff.
7. Apply for the HCSSA (PAS) license through TULIP; pay the fee; confirm the current amount in the HHSC fee schedule.
8. Pass the pre-survey / initial survey; correct any deficiencies promptly and fully.
9. Recruit, screen, train to competency, and schedule your first attendants.
10. Set up documentation and HHAExchange EVV before your first Medicaid visit.

11. For Medicaid: obtain your HHSC contract, enroll in TMHP/PEMS with the surety bond, and contract with STAR+PLUS MCOs.
12. Begin weekly referral outreach to discharge planners, case managers, and the aging network.
13. Stand up your QAPI log and supervisory-visit routine from week one.

Opening a home care agency is a real undertaking, but it is an achievable one — thousands of Texans have built good agencies from exactly this starting point. Take it one verified step at a time, build genuine systems rather than shortcuts, and lead with reliability. Do that, and you will be running a business your community needs and that can grow with you for years.

Final word

This guide is an educational resource, **not legal, tax, or billing advice**. Texas rules, fees, codes, rates, and contacts change — verify every step and figure with HHSC, TMHP, and the relevant payer or managed-care organization before you act.